Pediatric Drug Dosages Listed
P-3 Reviewed March 30, 2020

THIS MASTER LIST REFLECTS ONLY THE PROTOCOL AND DOSING FOR THE FOLLOWING FORMULARY OF MEDICATIONS CARRIED BY FFEMS. IT DOES NOT REFLECT ALL THE INDICATIONS, CONTRAINDICATIONS, PRECAUTIONS AND USAGE DIRECTIONS FOR THE MEDICATION. YOU MUST REFER TO THE PROTOCOL FOR THIS INFORMATION.

ADENOSINE

Protocol: C-8 SVT: Adult and Pediatric

STABLE PATIENT: AT M.D. DISCRETION ONLY: SVT: 0.1 mg/kg rapid IV bolus

UNSTABLE PATIENT: Adenosine 0.1 mg/kg rapid IV push followed by normal saline flush of 2-5 mL If there is no effect, the dose may be doubled. The maximum single dose of adenosine should not exceed 6 mg.

ALBUTEROL

Protocol: P5 - Pediatric Respiratory Distress

Age ≤ 2 years: ½ dose of Albuterol 2.5 mg (1.25mg) administered in a nebulizer.

Age > 2 years: Albuterol 2.5 mg administered in a nebulizer

AMIODARONE

Protocol: C-7 VF/VT Cardiac Arrest Adult and Pediatric

5mg/KG IV/IO bolus, may repeat at same dose once in 3-5 minutes

ATROPINE

Protocol: O-2 Suspected Overdose/Poisoning

Organophosphate poisoning: 0.02 mg/kg IV push, may be repeated

ATROVENT

Protocol: P-5 Pediatric Respiratory Distress/Failure

Age ≤ 10 years: **½ dose** 500 mcg of Atrovent (250mcg)

Age > 10 years: 500 mcg Atrovent

BENADRYL

Protocol: O-2 Suspected Overdose/Poisoning

Anti-psychotic poisoning: 1mg/Kg IM ONLY not to exceed adult dose

CALCIUM CHLORIDE:

Protocol: O-2 Suspected Overdose/Poisoning

Calcium Chloride is "AT MD DISCRETION ONLY"

Protocol: C-7 PEA/Asystole

"AT MD DISCRETION ONLY" Calcium Chloride 10% solution at 0.2 ml/Kg IV/IO. Flush IV line.

CYANOKIT

Protocol: O-2 Suspected Overdose/Poisoning

Cyanokit (Hydroxocobalamine) 70 mg/KG IVPB over 15 minutes into its own dedicated IV line. Maximum dose is 5 g.

50% DEXTROSE:

Protocol: M-3 Hypo/Hyperglycemia

For hypoglycemia in child > 2 years: 1ml/Kg IV Maximum 25 g per dose May repeat once in 5 minutes

25% DEXTROSE:

Protocol: M-3 Hypo/Hyperglycemia

For hypoglycemia in 1 month to 2 years: 2ml/Kg IV may repeat once in 5 minutes

10% DEXTROSE:

Protocol: M-3 Hypo/Hyperglycemia

5 ml/Kg IV Repeat once in 5 minutes

Protocol: OB-2 Newly Born Resuscitation

D10%W at 2 ml/Kg for hypoglycemia ≤ 40 mg/cl

EPINEPHRINE 1:1,000

Protocol: M-2 Allergic Reactions/Anaphylaxis

0.01 mg/Kg IM (not to exceed 0.15 mg) Repeat in 5 minutes **ONLY** if respiratory distress continues.

Protocol: P- 5 Pediatric Respiratory Distress

0.01mg/Kg IM Maximum single dose is 0.3mg

Protocol: P- 5 Pediatric Respiratory Distress

Administer 1.5 mg of a 1:1,000 with 3 ml saline nebulized

Protocol: O-2 Suspected Overdose/Poisoning

Chlorine/Hydrochloric Acid Vapor Exposure 1.5 ml epinephrine 1:1,000 in 3 ml of normal saline.

EPI PEN JR

Protocol: M-2 Allergic Reactions/Anaphylaxis

Children > 60 lbs. 0.3 mg IM Further dosing MUST notify Medical Control.

Children 25 lbs. to 60 lbs. 0.15 mg IM

Children < 25 lbs. CALL MEDICAL CONTROL

EPINEPHRINE 1:10,000

Protocol: C-2 Symptomatic Bradycardia Adult and Pediatric

0.01 mg/Kg or 0.1 ml/kg (1:10,000) IV Repeat the same dose every 3-5 minutes if pacing cannot get capture, be tolerated or fails to improve CO.

Protocol: C-5 PEA/Asystole Adult and Pediatric

Protocol: C-7 VF/Pulseless VT Adult and Pediatric

0.01mg/Kg or 0.1ml/Kg IV/IO every 3-5 minutes prn cardiac arrest

FENTANYL

Protocol: G-6 Pain Management Non Cardiac Procedural Analgesia

Dosing for child that cannot communicate is AT MD DISCRETION ONLY.

Others: 1 mcg/Kg IN or slow IVP If; MAXIMUM SINGLE DOSE 50 mcg Administer ½ of IN dose in each nostril May be repeated every 10 minutes

Pain Management for Synch Cardiovert and Pacing **AT MD DISCRETION ONLY**

Protocol G-10 Post Intubation Sedation and Pain Management

For agitation, Administer 1 mcq/Kg Fentanyl IV/IO (Maximum single dose 25 mcg). If B/P > fifth percentile This dosing regimen may be repeated every 5 minutes as needed.

GLUCAGON

Protocol: M-2 Allergic Reactions/ Anaphylaxis

0.1 mg/kg IM (Maximum 1 mg) Repeat once in 15 minutes if needed

KETAMINE

Protocol: G-8 Ketamine for Acute Pain Control

Administer **0.2mg/Kg ketamine** IV slow push over 90 seconds.

May repeat **at 0.2 mg/Kg ketamine** in 10 minutes if warranted

Protocol G-11 Post Intubation Sedation and Pain Relief

Ketamine 1 mg/kg IV/IO SLOWLY May be given INITIALLY. This dose may be repeated in 15min if needed one time.

LIDOCAINE 2%

Protocol: G-1 Alternate Venous Access Devices/IO Insertion

0.5 mg/Kg up to 20 mg IO slowly May repeat once

Protocol: O-2 Suspected Overdose/Poisoning

Tricyclic Antidepressant Overdose Lidocaine 1.0 mg/Kg IVP.

NARCAN

Protocol: OB/GYN 2 Newly Born Resuscitation

AT MD DISCRETION ONLY Initial dose is 0.1 mg/kg IV of a l mg/ml solution repeated in 2-3 minutes as needed. Avoid Narcan use in newborn whose mother is narcotic dependent. Do not use ETT Narcan in neonate.

Protocol: O-2 Suspected Overdose/Poisoning

0.1 mg/Kg IN with divided dose in each nostril OR naloxone 0.1 mg/Kg IV.

SODIUM BICARBONATE

Protocol: O-2 Suspected Overdose/Poisoning

Protocol: C-5 Pea/Asystole Adult and Pediatric

Protocol: C-7 VF/Pulseless VT Adult and Pediatric

TCA Overdose 1 meq/Kg IVP . (< 3 months of age use 4.2% sodium bicarbonate) Do not exceed 50 mEq.

Protocol: O-2 Suspected Overdose/Poisoning

Chlorine/Hydrochloric Acid Vapor Exposure: Nebulized sodium bicarbonate is "AT MD DISCRETION ONLY"

VERSED

Protocol: M-8 Seizure Adult and Pediatric

Versed 0.2 mg/Kg IM in the vastus lateralis, IV, IN ***Do not use IN ml dosing for IV dose.***

MAXIMUM 10 mg single dose IN

MAXIMUM 10 mg single dose IM

MAXIMUM 5 mg single dose IV

May repeat dosing every 5 minutes for continued seizure activity. Maximum cumulative dose 1 mg/Kg OR 20 mg.

Further dosing above maximum cumulative dose AT MD DISCRETION ONLY.

Protocol: G-8 Ketamine for Acute Pain Control

Laryngospasm, administer **0.2 mg/Kg midazolam IVP** every 5 minutes and prepare for aggressive management of the airway. Maximum single dose is 5 mg. Maximum cumulative dose is 1 mg/Kg or 20 mg.

Emergence Reaction**:** Administer **0.2 mg/kg midazolam IVP** over 2 minutes. Maximum single dose is 5 mg.

Protocol G-10 Post Intubation Sedation and Pain Control

Versed 0.2 mg/kg IV/IO (maximum 5 mg single dose) May repeat once in 10 minuites

ZOFRAN

Protocol: G-5 Pain Management Non Cardiac/ Procedural Analgesia

Protocol: M-1 Abdominal Pain

Protocol: M-4 Heat Emergencies

Protocol: G-8 Ketamine for Acute Pain Control

Zofran 0.1 mg/Kg IVP slowly as needed for nausea and vomiting. Single dose not to exceed 4 mg.